

Test Report

CLINICAL SUMMARY & DIAGNOSTIC REPORT

1. PATIENT INFORMATION

- **Full Name:** Johnathon D. Doe
- **Date of Birth:** 14 October 1982
- **Age / Gender:** 43 / Male
- **Patient ID:** PT-99201-A
- **Medical Record Number:** MRN-883011
- **Date of Examination:** 04 August 2026

2. CLINICAL OVERVIEW

- **Referring Physician:** Dr. Sarah Jenkins, MD (Internal Medicine)
- **Facility:** Metropolitan General Hospital, Outpatient Clinic
- **Chief Complaint:** Persistent fatigue, mild exertional dyspnea, and recurrent bilateral lower extremity edema over the past three weeks.
- **History of Present Illness:** Patient reports a gradual onset of lethargy despite adequate rest. Denies chest pain, palpitations, orthopnea, or recent viral illnesses.

3. PAST MEDICAL HISTORY

- **Chronic Conditions:** Hypertension (Diagnosed 2021), Hyperlipidemia (Diagnosed 2023)
- **Surgical History:** Laparoscopic Appendectomy (2018)
- **Allergies:** Penicillin (Reaction: Anaphylaxis / Urticaria)
- **Current Medications:**
 - Lisinopril 20mg – 1 tablet orally daily
 - Atorvastatin 10mg – 1 tablet orally at bedtime

4. PHYSICAL EXAMINATION & VITALS

- **Blood Pressure:** 138/86 mmHg
- **Heart Rate:** 74 bpm (Regular rhythm)
- **Respiratory Rate:** 16 breaths/min
- **Temperature:** 36.7 °C (98.1 °F)
- **Oxygen Saturation:** 98% on room air
- **Cardiovascular:** S1 and S2 distinct, no audible murmurs, gallops, or rubs.
- **Pulmonary:** Clear to auscultation bilaterally. No wheezes, rales, or rhonchi.
- **Extremities:** 1+ pitting edema noted bilaterally up to the mid-calf region.

5. LABORATORY RESULTS (COMPLETE BLOOD COUNT & METABOLIC PANEL)

Test Name	Result	Flag	Reference Range	Units
Hemoglobin	11.2	L	13.8 – 17.2	g/dL
Hematocrit	34.1	L	41.0 – 50.0	%
White Blood Cells (WBC)	6.8		4.5 – 11.0	x10 ³ /μL
Platelets	245		150 – 450	x10 ³ /μL
Serum Glucose (Fasting)	94		70 – 99	mg/dL
Serum Creatinine	1.42	H	0.70 – 1.30	mg/dL
Blood Urea Nitrogen (BUN)	26	H	7 – 20	mg/dL
Sodium	137		135 – 145	mEq/L
Potassium	4.2		3.5 – 5.1	mEq/L

Note: Flags indicate abnormality (**L** = Low, **H** = High).

6. CLINICAL IMPRESSION & DIAGNOSIS

- Mild Microcytic Anemia** – Likely secondary to chronic disease or nutritional deficiency; requires further iron studies evaluation.
- Stage 2 Chronic Kidney Disease (CKD)** – Indicated by a calculated eGFR of 58 mL/min/1.73m² alongside elevated Serum Creatinine and BUN levels.
- Essential Hypertension** – Currently stable, though borderline elevated on current therapy.

7. PLAN & RECOMMENDATIONS

- Diagnostic Workup:** Order a fasting iron panel (Serum Iron, TIBC, Ferritin) and a quantitative urinalysis with a microalbumin-to-creatinine ratio.
- Cardiology Referral:** Schedule a routine transthoracic echocardiogram (TTE) to evaluate left ventricular function given the new onset of peripheral edema.
- Dietary Modifications:** Initiate a low-sodium, kidney-friendly diet (<2,000 mg sodium daily). Maintain adequate hydration.
- Follow-up:** Return to the clinic in 2 weeks for a repeat basic metabolic panel (BMP) and medication review.

Provider Electronic Signature:

Dr. Sarah Jenkins, MD

License No: #MD-8840211

Document Electronically Signed on: 04 August 2026